

Discharge Summary

Patient Name: Melike Bengisu Demir
ID No: 11272627536
Place/Date of Birth: Alanya 02.02.1994
Father's Name: Hasan
Gender: Female
Institution: Social Security Institution
Admission Date: 20.10.2025 09:30
Ward: General Surgery Service 2
Doctor: Op. Dr. Mehmet Olcum
Address: Carsi Mah. Seral Sk. No: 30/2 Alanya/Antalya

Reason for Hospitalization, Diagnoses, and Comorbidities:
Final Diagnosis: E04.2 - Non-toxic multinodular goiter (Examination)
Final Diagnosis: E04.2 - Non-toxic multinodular goiter (Hospitalization)

Follow-up Information:
Reason for Admission / Complaints:
Palpable swelling in the neck - shortness of breath

History:
The patient was referred from endocrinology due to goiter with pressure symptoms. Tests showed a giant nodule on the left side. She was hospitalized for surgery.

Past Medical / Family History:
None.

Physical Examination:
Left thyroid lobe palpable.

Clinical Course / Daily Notes:
The patient was operated. No hoarseness, Chvostek negative. Drain output 50 cc. The patient was discharged on postoperative day 2 with recommendations.

Neurological Findings:
(Not specified)

Treatment:
Inpatient service visits on 20, 21, 23 October. IV injection and IV infusion on 22 October.

Laboratory:
Blood type A Rh(+), calcium values 8.2–8.5, PTH 4.6.

Used Materials:
IV sets, syringes, gloves, masks, etc.

Medications Given:
Anti-nausea medication, Levotiron, antibiotics, electrolyte solutions, Parol infusion, Zygosin, Calcimax D3, etc. (Full list in original document)

Operations Performed:
21.10.2025 - Bilateral total thyroidectomy
21.10.2025 - Cortical stimulation
Surgeon: Mehmet Olcum
Anesthesia: General anesthesia

Operation Duration: 160 minutes
Anesthesia Duration: 170 minutes

Operation Note:

Under ITGA, the neck was extended in Semi-Fowler position. A Kocher collar incision was made. Skin, subcutaneous tissue, and platysma were opened. Superior and inferior flaps were dissected. Thyroid anterior muscles were separated. Right superior thyroid vessels were ligated, then middle and inferior thyroid vessels. Recurrent laryngeal nerve and parathyroids were preserved. Right lobe was totally removed. The same procedure was repeated on the left side. Central lymph node dissection was performed. Hemovac drains were placed. The wound was closed anatomically.

Complications:

None reported.

Post-Discharge Instructions:

Use prescribed medications. Follow recommendations provided at discharge.

Companion Information:

Date: 20.10.2025 - Companion: Kamer Demir - ID: 11593617520

End of Summary.